



Department of Defense
Defense Health Agency
Performance Work Statement

Newborn Screening Services
Naval Medical Center Camp Lejeune, North Carolina

PART 1

1.0 GENERAL INFORMATION

1.1 **This is a non-personal services contract** to provide Newborn Screening Services in support of Naval Medical Center Camp Lejeune, North Carolina.

1.2 **Description of services/introduction:** The contractor shall provide non-personal services of Newborn Screening to patients in accordance with the terms and conditions defined in this Performance Work Statement (PWS) at the Naval Medical Treatment Facility (MTF) indicated in the awarded contract.

1.3 **Background:** Newborn Screening is recognized nationally as an essential preventive public health measure. Newborn screening is the practice of testing all babies for certain disorders and conditions that can hinder their normal development. Babies with these conditions appear healthy at birth but can develop serious medical problems later in infancy or childhood. Early detection and treatment can help prevent intellectual and physical disabilities and life-threatening illnesses. Base contract N6264521D5004 was awarded to Health and Human Services, North Carolina, Public Health State Laboratory on 01 May 2021 to provide Newborn Screening Services to Naval Medical Center, Camp Lejeune, North Carolina. The period of performance for N6264521D5004 is from 01 May 2021 through 30 April 2026. As part of the Department of Defense healthcare system, Military Treatment Facilities with Labor and Delivery (L&D) units are required to comply with military, federal, and state public health regulations regarding newborn screening. Ensuring early detection of congenital and metabolic disorders is critical for readiness and the long-term health of military families.

1.4 **Scope:** The contractor shall provide Newborn Screening Services in support of Naval Medical Center Camp Lejeune, North Carolina. The Government reserves the right to add additional Military Treatment Facilities where North Carolina State services may be required, as necessary, via bilateral modification. Services include the supply of blood specimen collections kits, laboratory analysis of dried blood spot samples, and timely electronic reporting of results. Services must cover all conditions required by the Department of Health and Human Services Recommended Uniform Screening Panel (RUSP). If applicable, additional testing such as biochemical and molecular/genetic tests should be performed to clarify screening results, according to the contractor's established protocols.

1.5 **Period of Performance (PoP):** The contractor shall begin providing services per the Performance Work Statement on 01 May 2026 through 30 April 2027.

Option Period 1: 01 May 2027 through 30 April 2028

Option Period 2: 01 May 2028 through 30 April 2029

Option Period 3: 01 May 2029 through 30 April 2030

Option Period 4: 01 May 2030 through 30 April 2031

Option Period 5 (-8, Six Month Option): 01 May 2031 through 31 October 2031

1.6 Administrative Specifications

1.6.1 **Place of performance:** The contractor shall perform newborn screening testing services through its designated reference laboratory.

Specimens collected by the Military Treatment Facility - Naval Medical Center, Camp Lejeune shall be received for initial processing and forwarded to the contractor's designated reference laboratory for newborn screening testing. The Government reserves the right to add additional Military Treatment

Facilities where North Carolina State services may be required, as necessary, via bilateral modification. All testing and result reporting shall be performed at the reference laboratory located at:

North Carolina Department of Health and Human Services
4312 District Drive
Raleigh, NC 27607-5490

1.6.2 Recognized Federal holidays: The contractor is not required to conduct business on Federally recognized holidays or Presidential Executive Orders.

1.6.3 Hours of operation: The contractor is responsible for conducting business Monday through Friday except Federal holidays or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings. The contractor must at all times maintain an adequate workforce for the uninterrupted performance of all tasks defined within this PWS when the Government facility is not closed for the above reasons.

1.7 Quality

1.7.1 Quality Control (QC): The contractor shall develop and maintain an effective QC program designed for medical surveillance to continuously monitor and improve quality of specimen collection, submission of required demographic information from submitting facilities, specimen analysis, and reporting of results in accordance with this PWS. Quarterly review or report to be provided. The contractor shall develop and implement procedures to identify, prevent, and ensure nonrecurrence of defective services. The contractor's QC program is the means by which the work complies with stated requirements. The QCP is to be delivered with the contractor's proposal and comprehensive written QCP shall be submitted to the CO and COR within 5 working days when changes are made thereafter.] After acceptance of the Quality Control Plan (QCP) the contractor shall receive the CO's acceptance in writing of any proposed change to his QC system. See Part 7, Technical Exhibit 1 - CDRL A001.

1.7.2 Quality assurance (QA): The government will evaluate the contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP). This plan provides a systematic method for the Government to evaluate performance and to ensure that the contractor has performed in accordance with the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and the minimum acceptable defect rate(s).

1.8 Reporting

1.8.1 Government's COR: The COR monitors all technical aspects of the contract and assists in contract administration. The COR is authorized to perform the following functions: assure that the contractor performs the technical requirements of the contract; perform inspections necessary in connection with contract performance; maintain written and oral communications with the contractor concerning technical aspects of the contract; issue written interpretations of technical requirements, including Government drawings, designs, specifications; monitor contractor's performance and notifies both the CO and contractor of any deficiencies; coordinate availability of government furnished property; and provide site entry of contractor personnel. A letter of designation issued to the COR, a copy of which is sent to the contractor, states the responsibilities and limitations of the COR, especially with regard to changes in cost or price, estimates or changes in delivery dates. The COR is not authorized to change any of the terms and conditions of the resulting contract.

1.8.2 Post award conference/periodic progress meetings: The contractor agrees to attend any post award conference convened by the contracting activity or contract administration office in accordance with FAR Subpart 42.5. The CO, COR, and other Government personnel, as appropriate, may meet periodically with the contractor to review the contractor's performance. At these meetings the CO will apprise the contractor of how the government views the contractor's performance and the contractor will apprise the Government of problems, if any, being experienced. Appropriate action shall be taken to resolve outstanding issues. These meetings shall be at no additional cost to the government.

1.9 Contractor Identification

1.9.1 Contractor personnel performing services in a contractor capacity in a Government facility are required to possess and wear an identification badge that displays his or her name and the name of their company. All contractor personnel shall identify themselves as contractor support personnel in all forms of communication with all entities with whom DHA/Deputy Assistant Director for Acquisition (DADA)/Head of the Contracting Activity (HCA) has business dealings. The contractor shall: Answer all telephone calls and have a personalized voice message with an introductory statement that includes the fact that the person is contractor support personnel. Ensure all those with whom the person interacts in any face-to-face dealings while supporting the DAD-A understands that the person is contractor support personnel. Include a title block in all emails that states the fact that the person is contractor support personnel. Ensure all those with whom the person interacts in any face-to-face dealings while supporting DHA/DAD-A/HCA understands that the person is contractor support personnel.

1.9.1 Contractor personnel will be required to attend meetings or otherwise communicate with Government and/or other contract representatives to meet the requirements of this order. Contractor personnel shall make their contractor status known during introductions.

1.9.2 Contractor personnel, while performing in a contractor capacity, are prohibited from using their retired or reserve component military rank or title in any written or verbal communications associated with the contracts in which they provide services.

1.11. Personnel Security

1.11.1 The contractor shall comply with DoD 8570.01-M, "Information Assurance Workforce Improvement Program, CH4" November 10, 2015, as amended; 8500.01, "Cybersecurity", dated March 14, 2014; DoD Manual (DoDM) 6025.18, "Implementation of the Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule Compliance in DoD Health Care Programs" dated March 3, 2019, Department of Defense Instruction (DoDI) 6025.18 "HIPAA Privacy Rule Compliance in DoD Health Care Programs", dated March 13, 2019; and DoDM 5200.02 "Procedures for the DoD Personnel Security Program (PSP)," incorporation change 3, effective September 24, 2020. Contractor responsibilities for ensuring personnel security include, but are not limited to, meeting the following requirements:

1.11.1.1 Initiate, maintain, and document personnel security investigations appropriate to the individual's responsibilities and required access to Controlled Unclassified Information (CUI).

1.11.1.2 DHA Personnel Security Office does not deny any access to any automated information system (AIS), network, or Controlled Unclassified Information (CUI). If a contractor receives an unfavorable

background investigation, the request for access will be sent back to the FSO for further action. Any unfavorable adjudication will result in DHA Personnel Security Office not signing off on any access request.

PART 2

2.0 DEFINITIONS, ACRONYMS, AND APPLICABLE PUBLICATIONS/INSTRUCTIONS

2.1 Definitions:

2.1.1 Category D: Information Technology (IT) and Telecommunications Services (called D-Services)

2.1.2 Category R: Support (Professional/Administrative/Management) Services (called R-Services)

2.1.3 Contracting Officer (CO): A person with the authority to enter into, administer, and/or terminate contracts and make related determinations and findings.

2.1.4 Contracting Officer's Representative (COR): An individual, including a contracting officer's technical representative (COTR), designated and authorized in writing by the CO to perform specific technical or administrative functions. This individual does NOT have authority to change the terms and conditions of the contract.

2.1.5 Nonpersonal services contract: a contract under which the personnel rendering the services are not subject, either by the contract's terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees.

2.1.6 Quality Assurance Surveillance Plan (QASP): An organized written document specifying the surveillance methodology to be used for surveillance of contractor performance. The Government may either prepare the QASP or require the offerors to submit a proposed quality assurance surveillance plan for the Government's consideration in development of the Government's plan.

2.2 Acronyms:

AIS	Automated Information System
APL	Approved Products List APL
AQL	Acceptable Quality Level
ARRT	Acquisition Requirements Roadmap Tool
ATO	Authority to Operate
B2B	Business-2-Business
CAC	Common Access Card
CAP	Cloud Access Point
CCEVS	Common Criteria Cybersecurity Evaluation and Validation Scheme
CDI	Covered Defense Information
CE	Computer Environment
CDRL	Contract Data Requirement List
CIO	Chief Information Officer
CJCSM	Chairman of the Joint Chiefs of Staff Manual

CMMC	Cybersecurity Maturity Model Certification
CMR	Contractor Manpower Reporting
CNSSI	Committee on National Security Systems Instruction
CO	Contracting Officer(s)
CONUS	Continental United States (excludes Alaska and Hawaii)
COR	Contracting Officer Representative
COTR	Contracting Officer's Technical Representative
CSP	Cloud Service Provider
CSSP	Cyber Security Service Provider
CUI	Controlled Unclassified Information
DAD-A	Deputy Assistant Director for Acquisition
DC3	DoD Cyber Crime Center
DD Form 254	Department of Defense Contract Security Requirement List (if applicable)
DB	Design-Build
DBB	Design-Bid-Build
DFARS	Defense Federal Acquisition Regulation Supplement
DHA	Defense Health Agency
DISA	Defense Information System Agency
DoD	Department of Defense
DoDD	Department of Defense Directive
DoDI	Department of Defense Instruction
DSAs	Data Sharing Agreements
DSAA	Data Sharing Agreement Application
DMZ	Demilitarized Zone
DoDM	Department of Defense Manual
DPCLO	DHA Privacy and Civil Liberties Office
DUA	Data Use Agreement
eMSM	Enhanced Multi-Service Markets
EULA	End User License Agreement
EVM	Earned Value Management
FAR	Federal Acquisition Regulation
FCI	Federal contract information
FE	Facilities Enterprise
FedRAMP	Federal Risk Authorization and Management Program
FISMA	Federal Information Security Modernization Act
FRCS	Facility Related Control Systems
FSO	Facilities Security Officer
HA	Health Affairs
HIPAA	Health Insurance Portability and Accountability Act
HCA	Head of the Contracting Activity
HIT	Health Information Technology
IGCE	Independent Government Cost Estimate
IA	Information Assurance
IO	Initial Outfitting
I/O	In/Out Processing Portal
IPv	Internet Protocol Version

IS	Information System
ISP	Internet Service Provider
IT	Information Technology
ISCM	Information Security Continuous Monitoring
IV&V	Independent Verification & Validation
MedCOI	Medical Community of Interest
MHS	Military Health System
MIL-STD	Military Standard
MTFs	Military Treatment Facilities
NCR	National Capitol Region
NDA	Non-Disclosure Agreement
NIAP	National Information Assurance Partnership
NIST	National Institute of Standards and Technology
OCONUS	Outside Continental United States (includes Alaska and Hawaii)
ODC	Other Direct Costs
OPM	Office of Personal Management
OSD	Office of the Secretary of Defense
P-ATO	Personal Authorization to Operate
P&R	Personnel and Readiness
PGI	Procedures, Guidance and Information
PDT	Project Delivery Team
PHI	Protected Health Information
PII	Personally Identifiable Information
PIT	Platform Information Technology
PK	Public Key
PKI	Public Key Infrastructure
POA&M	Plan of Action and Milestones
POC	Point of Contact
PMO	Program Management Office
PoP	Period of Performance
PP	Personal Property
PPSM	Ports, Protocols, and Services Management
PRS	Performance Requirements Summary
PSP	Personnel Security Program
PWS	Performance Work Statement
QA	Quality Assurance
QAP	Quality Assurance Program
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Plan
RFP	Request for Proposal
RFQ	Request for Quotation
RMF	Risk Management Framework
SP	Special Publication
SPRS	Supplier Performance Risk System
SRM	Sustainment, Restoration and Modernization
SRG	Security Requirements Guides

STIG	Security Technical Implementation Guides
TOS	Terms of Service
US	United States
UFC	Unified Facilities Criteria
VPN	Virtual Private Network
XML	Extensible Markup Language

PART 3

3.0 GOVERNMENT FURNISHED PROPERTY, EQUIPMENT, AND SERVICES

The Requiring Activity Authority has assessed the need for Government Furnished Property, Equipment, and Services and determined:

3.1 Services:

The Government:

☒ Will **NOT** provide Government Furnished Services in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Government Furnished Services required in support of this contract/task orders. These Services are described below: Not applicable.

3.2 Facilities:

The Government:

☒ Will **NOT** provide Facilities in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Facilities in support of this contract/task orders. The Government provided Facilities are described below: Not applicable.

3.3 Utilities:

The Government:

☒ Will **NOT** provide Utilities in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Utilities in support of this contract/task orders. The Government provided Utilities are described below: Not applicable.

3.4 Equipment:

The Government:

☒ Will **NOT** provide Equipment in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Equipment in support of this contract/task orders. The Government provided Equipment is described below: Not applicable.

3.4.1 **Procurement Integrated Enterprise (PIEE), GFP Module Application.** Not applicable.

3.5 Materials:

The Government:

☒ Will **NOT** provide Materials in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **IS** providing Materials in support of this contract/task orders. The Government-provided Materials are described below: Not applicable.

PART 4

4.0 CONTRACTOR FURNISHED ITEMS AND SERVICES

4.1 Services:

The Contractor:

☒ Will **NOT** provide Contractor Furnished Services in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Contractor Furnished Services required in support of this contract/task orders. These Services are described below: Not Applicable.

4.2 General:

The contractor shall furnish all supplies, equipment, facilities and services required to perform work listed under Section 5 of this PWS.

4.3 **Materials:** The contractor shall provide a specimen collection kit and a special filter paper card in accordance with the contractor's standard practice.

4.4 **Equipment:** Not applicable.

4.5 Facilities: The contractor shall store residual dried blood spots at a facility that provides security, confidentiality, stability of temperature and humidity, and retrievability for 5 years.

PART 5

5.0 SPECIFIC TASKS

5.1. Specimen Collection Material Supply and Specimen Transport.

5.1.1. The contractor shall provide a specimen collection kit and a special filter paper card in accordance with the contractor's standard practice.

5.1.2. The MTF will provide the contractor with the filter paper card collected from all newborns by heel-stick between 24-48 hours of age, but not greater than the 4th day of life unless the clinical situation makes later collection necessary. If collected prior to 24 hours, the facility shall submit a second specimen in accordance with the contractor's standard practice.

5.1.3. The MTF will complete the test request form (TRF), which contains fields for recording critical demographic information about the baby.

5.1.4. The MTF will register with and use the North Carolina Hearing Link. The Hearing Link, already installed at this facility, is a program for entering newborn demographic information, details about collection of the newborn screening dried blood spot specimen and details about newborn hearing screening. The Hearing Link will print labels with such demographic information which are applied to the newborn screening filter form.

5.1.5. Transport of specimens collected and to be tested by the contractor will be shipped by the MTF in accordance with the contractor's standard practice.

5.1.6. The contractor shall retain dried blood spots in accordance with section 5.7.3 below.

5.2 Training.

5.2.1. Prior to submission of samples and as requested by the MTF during the life of this contract, the contractor shall provide the MTF staff with training, education and technical assistance regarding the proper sample collection process, sample shipment preparation and packaging, and logistical coordination for sample processing. Such training, education, or technical assistance may be provided in person, by phone, e-mail, webcam, video teleconference, videotape, or letter, as appropriate, to improve quality of specimen submission, decrease sample collection and patient identification errors, and reduce the need for newborn re-screens

5.2.2. The contractor shall not be compensated separately for training.

5.3. Screening Tests.

5.3.1. The contractor shall provide the MTF Newborn screening services to include laboratory testing for the detection of a core set of conditions and report results for additional disorders detected by this testing. The conditions to be tested shall include those on the Recommended Uniform Screening Panel (which is

hereby incorporated as reference), as recommended by the Secretary's Advisory Committee on Heritable Disorders in Newborns and Children with the approval of United States Department of Health and Human Services and as approved by state law. Any conditions that are added to the Recommended Uniform Screening Panel throughout this contract period shall be added to the set of conditions screened under this contract as they are able to be incorporated and validated by the contractor. The contract may be modified if the cost of screening increases to include those conditions and or/if testing technologies change and/or improve.

5.3.2 Unless enactment of legislation changing the rate does not allow adequate time, the MTF shall be given a minimum of 30 days advance notice of a change of testing (Addition/Deletion/Modification of screened disorders and/or screening technologies). Contractor notification will consist of (at the minimum) an official letter, sent on contractor official departmental letterhead to the Contracting Officer, addressed to the MTF laboratory medical director and laboratory administrator. Additional disorders may be added as they are incorporated into the state's Newborn Screening program and/or new disorders become prevalent. Changes in testing methodology may occur without notice, such as the addition or elimination of ratios, changes in analyte cut-offs, etc.

5.4 Laboratory Testing and Analysis.

5.4.1. The contractor shall provide newborn screening for all of the disorders that the contractor provides in its State- wide Newborn Screening Panel of disorders. In addition, when appropriate for initial screening or secondary testing, biochemical and molecular/genetic tests should be performed to clarify screening results, according to the contractor's established protocols.

5.4.2. Repeat testing and/or confirmatory tests on the initially collected patient specimen shall be performed as appropriate if a test does not fall within normal limits on the initial screening test according to the contractor's established protocols.

5.4.3. The contractor shall provide re-screening of newborns in cases of improper sample collection, presumptive positive screening results, technical problems such as patient identification errors or degradation of specimens during shipment/transport.

5.4.4. The contractor shall provide re-screening of all infants weighing less than 1,500 grams at birth, and premature and sick infants requiring prolonged hospitalization according to the contractor's established protocols and not separately invoiced. Of these, the contractor shall provide re-screening in cases where initial results are presumptively positive.

5.5 Case Management for Screens.

5.5.1. The contractor shall perform the required tests and then provide the results of the screen by electronic post to authorized personnel at the MTF. Results should be reported within 5 business days, but not longer than 7 business days of sample receipt. As a last resort for notification of results, copies of the results may also be mailed via United States Postal Service (USPS) to the physician of record or the ordering hospital according to the contractor's standard practice and as agreed upon by the MTF.

5.5.2. Timelines and methods of delivery of presumptive positive/significant abnormal results will follow the contractor's established protocols.

5.5.3. Abnormal results of an urgent nature will be reported by phone, as well as by secure email, electronic post, or as a last resort be mailed via USPS, to the MTF newborn screening coordinator and the health care provider (as indicated on the screening card or by medical records at the MTF) by the contractor's designated Newborn Screening Program Follow-up Specialists. Appropriate recommendations for further testing or follow-up will be made.

5.5.4. For abnormal results of an urgent nature, the contractor's designated Newborn Screening Program Follow-up Specialists shall monitor the newborn's case until screening results are reported or diagnosis is confirmed.

5.6 Specialty Care for Positive Screens.

5.6.1 Specialist in metabolic genetics, immunology and hematology will be available for consultation by phone or email to advise patient management, if required. Contact information for Specialist in pulmonology or endocrinology will be made available if indicated.

5.6.2 Specialists aid/or the contractor's Newborn Screening Follow-up Program coordinators will assist in establishing appropriate follow up of screen positive babies.

5.6.3 The contractor shall be readily available for any questions by the MTF Healthcare Providers that may not have been covered in the Laboratory testing and provide quality customer service herein to the terms of the intent and terms of this contract.

5.6.4 The contractor shall contact the respective MTF laboratory to ensure a positive transfer of patient results (for all results, both normal and abnormal).

5.6.5 The Medical Record Technician will record the Newborn Screen results into the appropriate Medical Records database

5.6.6 As part of continued follow-on care, the communication between the contractor and the MTF shall continue regularly without prompting from the MTF.

5.6.7 The contractor shall not be compensated separately for Specialty Care for Positive Screens.

5.7. Other Responsibilities.

5.7.1. The contractor shall keep the Contracting Officer Representative (COR) informed of the most recent developments in clinical laboratory methods and medical interpretation through periodic newsletters, reports and data sheets.

5.7.2. The contractor shall have reference laboratory representatives available to assist in problem solving at no additional cost to the Government with access to knowledgeable laboratory professionals with minimal call transfers and holding.

5.7.3. Residual dried blood spots shall be stored at a facility that provides security, confidentiality, stability of temperature and humidity, and retrievability for 5 years. Stored specimens remain the property of the Department of Defense (DoD) throughout the storage period. The contractor shall retrieve and provide stored specimens to the DoD within 30 days upon request using contractor's established protocols and with written authorization from the parent/guardian. If it is the contractor's practice to dispose of filter paper blood spots following the 5-year period, the left-over filter paper blood spots shall be disposed of, and it

shall be in a manner that ensures confidentiality. No filter paper blood spots can be used for research without the explicit consent of the newborn's parent/legal guardian, and approval by a DoD approved Institutional Review Board. Documentation of the storage, use and disposal of residual dried blood spots shall be maintained and shall be made available to the DoD upon request.

5.8. Volume. Quantity amounts provided for CLINs are estimated based on historical data and is subject to fluctuation. The Government reserves the right to reevaluate quantities, if necessary.

5.9 Invoicing.

5.9.1 The contractor shall submit invoices every month. The cycle is from the first of the month and through the end of the month and shall not coincide with any other dates.

5.9.2 In the event that repeat testing or re-screening is necessary due to abnormalities etc., the contractor shall bill on the first test.

PART 6

6.0 INFORMATION TECHNOLOGY & SECURITY

6.1 All work under this contract is unclassified.

6.2 The TIER 1 or TIER 2 levels and position sensitivity designation for positions under this contract is: Reserved.

6.3 Personally Identifiable Information (PII)/Protected Health Information (PHI), Procurement, and Federal information requirements: Reserved.

6.4 Training: Reserved.

6.5 Cybersecurity Requirements for Non-DoD IT or Covered Contractor IS: Reserved.

6.6 Risk Management Framework (RMF) for DoD IT: Reserved.

6.7. Facility Related Control Systems: Reserved.

6.8. System Communications: Reserved.

PART 7, ATTACHMENT 1 PERFORMANCE REQUIREMENTS SUMMARY

The contractor service requirements are summarized into performance objectives that relate directly to mission essential items. The performance threshold briefly describes the minimum acceptable levels of service required for each requirement. These thresholds are critical to mission success.

Performance Objective (The Service required—usually a shall statement from Part 5)	PWS Reference	Acceptable Quality Level (AQL) (This is the maximum error rate. It could possibly be “Zero deviation from standard”)	Method of Surveillance
Performance Requirements Summary (PRS) # 1. The contractor shall ensure that material supplied be acceptable for testing.	5.1, 5.7.3	100% of the Performance Period	Monthly Inspection by COR/ACOR
PRS # 2 The contractor shall provide training to ensure acceptable standards for testing.	5.2	100% of the Performance Period	Monthly Inspection by COR/ACOR
PRS # 3 The contractor shall provide notice at least 30 days in advance to the military treatment facility for any testing changes.	5.3.2	100% of the Performance Period	Monthly Inspection by COR/ACOR
PRS # 4 The contractor shall provide the MTF Newborn screening services to include laboratory testing for the detection of a core set of conditions and report results for additional disorders. Tests should be for all disorders listed with rescreening provided as necessary.	5.3.1	100% of the Performance Period	Monthly Inspection by COR/ACOR
PRS # 5 Test Results Timeline	5.5.1	100% of the Performance Period	Monthly Inspection by COR/ACOR
PRS # 6 Test Results Method of delivery	5.5.1, 5.5.3	100% of the Performance Period	Monthly Inspection by COR/ACOR

PART 7, TECHNICAL EXHIBIT 1
DELIVERABLES SCHEDULE

<u>Deliverable</u>	<u>Frequency</u>	<u>Medium/Format</u>	<u>Submit To</u>	<u>Applicable to:</u>
CDRL A001 – Quality Control Plan  CDRL 1 Quality Plan	With Contractor's Proposal	Contractors discretion	CDRL A001 to CO	All, See PWS 1.7.1